AMBULANCE OPERATIONS

UNIT 3, PART 1: CTAS Introduction

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Canadian Triage & Acuity Scale
for Paramedics

Objectives

On completion of the program, the EMA/Paramedic will be able to:

- describe the Canadian Triage and Acuity scale, its application in the prehospital setting,
- perform an assessment on a patient and apply an appropriate acuity level to the patient using the CTAS acuity scale and,
- explain the reasons for assigning the CTAS level chosen.

Rationale for using CTAS

- standardized system of assigning acuity
- common to both ED and prehospital
  "speaking the same language"
- identify needs of patients – Improved patient care
- assist in determination of destination
- better tracking of patients and matching to facilities

CTAS

- assigns level of ***acuity***
- based on chief complaint as it relates to “sentinel events”
- Ensures that critically ill or injured receive priority attention (all parties
- ***patient focused <u>not</u> resource based***
- 5 levels that define optimal time to treatment in the ED

Application of CTAS in the Prehospital Setting:

- ***applied at contact, departure, arrival****

based upon:

»initial assessment

»clinical findings

»response to treatment

* Departure CTAS is the one we use for notifying dispatch

CTAS in Prehospital Setting

- similar in intent to “case severity” (our code 1-4)
- **not** intended to:
  - » replace return priority code
  - » nor affect dispatch priority code

CTAS in Prehospital Setting

return priority code refers to:

- the urgency of response or transport
- other use of an ambulance when a patient is not carried
- “poor proxy for acuity”
- will be reported to dispatch but no longer used in reporting to the ED

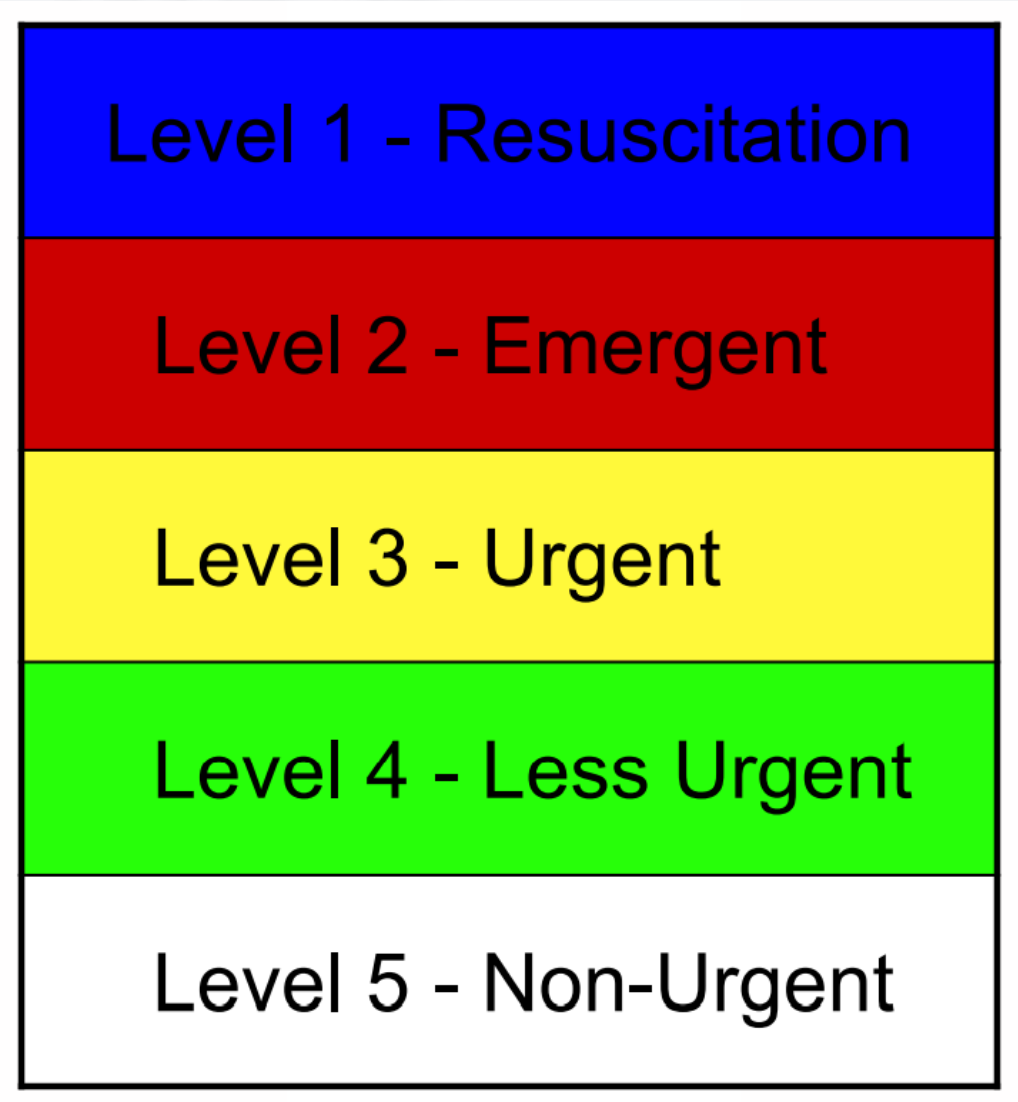
Levels of Acuity

5 levels of acuity:

Level 1 being the sickest patients

Level 5 being the least ill

Note: this is opposite the ambulance priority codes



Assigning a Level

Rule 1:

A minimum of 2 CTAS scores will be applied to each patient .

- At first patient contact
- Departing the scene (post assessment and initial interventions)

Assigning a Level

Rule 2:

The CTAS level reported to the receiving facility is the level assigned at scene departure OR of the patient deteriorates after transport has been initiated

Assigning a Level

Rule 3:

When taking into consideration the patient's response to treatment the acuity level assigned should not be any greater than 2 levels below the pre treatment acuity. (This is the initial first contact or arrival CTAS)

Assigning a Level

Example:

Unconscious diabetic GCS 6, blood sugar 1.2, given glucagon and responds to treatment. At transport GCS 15, alert, talking, normal vital signs.

- CTAS 1 on contact. CTAS 3 on departure

*CTAS in Prehospital Setting: Assigning a Level

- 24 year old male MVC requires needle decompression of a tension pneumothorax and improves prior to transport.
- CTAS 1 on contact. CTAS 2 on departure

Assigning a Level

Should the patient's condition deteriorate, it is important to notify the hospital of the change in the patient's status and acuity level.

Assigning a Level

Rule 4: For a patient who is VSA on arrival and who is resuscitated, the CTAS must stay as a CTAS 1

Assigning a Level

Rule 5: If paramedics receive a Termination of Resuscitation (TOR) order while managing a patient, the CTAS level assigned for the patient and documented on the ACR is based on the status of the patient on arrival and departure (if applicable) from the scene.

if code 5, report
CTAS 0

Assigning a Level

Rule 6: In cases where it is determined on arrival that a patient is “obviously dead”, no CTAS level (Arrival or Departure) is required to be assigned and documented as a zero (0) on the ACR.

Patient Priority System: Dispatch Destination Decisions

- Status of the hospitals
- The medical care that the paramedic feels the patient will require
- and, the CTAS level of acuity assignment:
  - level 1 – nearest/closest most appropriate facility (Except applicable “by-pass”).
  - level 2 - nearest/closest appropriate facility based on “communication” between paramedics and dispatch, and dispatch and the receiving ED and based on local rules.
  - level 3, 4, 5 –most available facility based on communication between dispatch and the receiving ED. These lower acuity levels may also take into consideration the patient’s wishes for destination.

Contents of the Patch

- Unit; e.g. “this is Simcoe unit 3182 en route to your facility……CTAS ___ with a …..
- patient age and sex
- chief complaint with brief HPI
- Any pertinent PMHx
- abnormal findings and abnormal vital signs
- treatment given and response to tx
- ETA

IT is not necessary to report ambulance priority code (i.e. Code 4)

QUESTIONS